

PRECISION CARE CHALLENGE 2026 · PHASE 1, IDEA SUBMISSION

# ClaimCast

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Know what your hospital bill will actually cost you. **Before** you are admitted, not **after** you are discharged.

**TRACK: HOSPITALITY** · Holistic Optimization System for Policy-Integrated Admission & Treatment Intelligence

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## Team Rocket

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# Explain The Problem You Are Attempting to Solve

**India's most expensive healthcare decision is made in ninety seconds, at an admission counter, by someone who has never read their policy.**

*IRDAI's chairman has said claim payouts often fall short of what policyholders expect.*

**AJAY SETH, CHAIRMAN, IRDAI · NOV 2025**

The regulator has named the gap. Policy bots explain the wording; none of them price the decision in front of you.

Proportionate deduction · IRDAI/HLT/REG/CIR/151/06/2020 · maximal case; scope varies by policy wording

## THE ROOM-RENT TRAP

**50%** of the bill can be refused

Your policy caps the room at **₹5,000** a day. The hospital gave you one at **₹10,000**.

Surgeon, OT, anaesthesia, nursing: **every charge your policy ties to room category** is scaled down by that same ratio.

**₹39,085**

mean out-of-pocket per cancer hospitalisation

12,148-patient multi-centre Indian study

**₹3.31 L**

out-of-pocket per cancer patient, per year

diagnostics 36% · medicines 45%

Cost shock drives distress financing, delayed care and outright **treatment abandonment**. It decides whether treatment finishes.

# Briefly Explain Your Idea/Solution

**Insurers compute what you are owed **after discharge**. ClaimCast computes it **before admission**.**

Same arithmetic. Different timestamp. That is the whole idea.

## 1 · ADMISSION

### Which hospital

network or not

**₹1.2 L**

at stake

## 2 · ADMISSION

### Which room class

proportionate deduction

**₹78,400**

at stake

## 3 · INVESTIGATION

### Cashless or reimburse

what you must float

**₹2 L**

at stake

## 4 · PROCEDURE

### Implant / consumable

sub-limits & List I

**₹45,000**

at stake

## 5 · RECOVERY

### 24-hour admission rule

day-care vs in-patient

**₹22,000**

at stake

### THE CARE JOURNEY THE PROBLEM STATEMENT NAMES

we price the decision at every stage, and each fork closes permanently the moment it passes

### MATERIALITY FILTER · OUR ANSWER TO FEATURE BLOAT

We compute the money at stake and **stay silent unless it clears ₹10,000**.

A caregiver at 2 a.m. sees only what is worth their attention.

### IS THIS UNIQUE, OR AN IMPROVISATION?

**Unique.** Existing tools **retrieve** policy text. ClaimCast **simulates the claim and optimises the decision**. Retrieval → Simulation.

**YOU WALK IN KNOWING WHAT YOU WILL OWE.** Same admission, ₹78,400 less out of pocket, because the caregiver was told before the decision closed.

# Explain Your Solution

We cannot tell you exactly what your bill will be. We can tell you exactly how much of it your policy will refuse.

AI where uncertain.

Arithmetic where known.

The LLM never decides the money.

## TWO HALVES, DELIBERATELY SEPARATED · PLUS A SOLVER

### A · FORECAST

Predicts the itemised bill from published tariff data. Uncertain, so it returns a range.

### B · ADJUDICATION

Computes what the policy refuses, line by line. Exact, with every ₹ cited to its clause.

### C · COUNTERFACTUAL

Re-runs the claim for every legal alternative and ranks them by what you keep.

## AGAINST THE CONVENTIONAL ALTERNATIVES

|          | Hospital / TPA desk | Policy bots · Ditto/PB | ClaimCast                  |
|----------|---------------------|------------------------|----------------------------|
| Speaks   | after admission     | any time, no context   | <b>at each decision</b>    |
| Returns  | a verbal estimate   | a text summary         | <b>itemised ₹ + clause</b> |
| Acts for | the hospital        | no one                 | <b>you</b>                 |

### PROS

- Deterministic and auditable
- Runs entirely on public data
- Rule-packs: private, PM-JAY, ESI

### CONS

- Forecast bounded by published tariffs
- Needs the policy document up front
- An estimate, never a guarantee

ClaimCast DECISION 2 OF 5 · ROOM CLASS · STAGE: ADMISSION

PROTOTYPE SCREEN

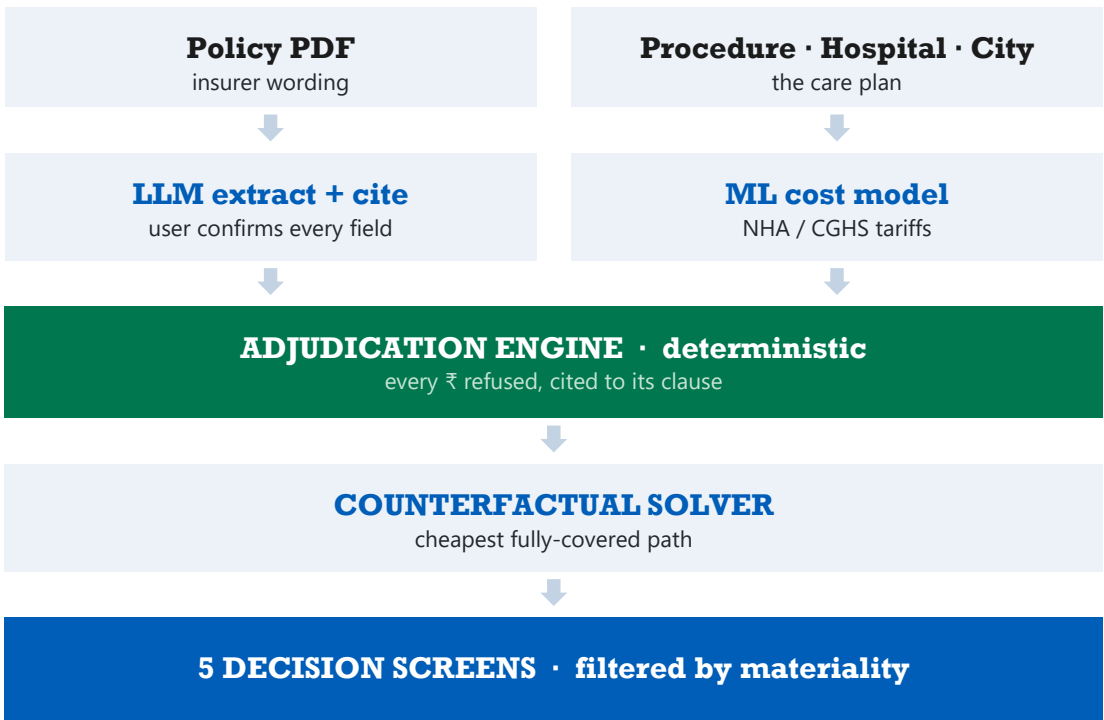
- Private room ₹10,000 / day your share **₹1,26,900**
- Semi-private ₹5,000 / day your share **₹48,500**

← recommended by ClaimCast

✓ **You keep ₹78,400** Room Rent Sub-limit, Clause 3.2 · proportionate deduction on the associated charges

# Any Additional Information / References

## ARCHITECTURE



## WHAT WE BUILD, AND WHAT WE SIMULATE

|                                      |          |
|--------------------------------------|----------|
| Policy → constraint parser           | Build    |
| Adjudication + deduction engine      | Build    |
| Cost model on NHA / CGHS rates       | Build    |
| Counterfactual solver + screens      | Build    |
| Live hospital tariff feed            | Simulate |
| Insurer cashless API / HIS bill feed | Simulate |

The two simulated feeds need an insurer and a hospital to open them, not more engineering.

## DATA WE SOURCE OURSELVES · ALL PUBLIC

NHA / PM-JAY Health Benefit Packages (≈1,900 procedure rates) · CGHS city-wise rate lists · IRDAI List I non-payables · IRDAI standard wordings (Arogya Sanjeevani) · NHA empanelled-hospital registry

## REFERENCES

- IRDAI, Circular IRDAI/HLT/REG/CIR/151/06/2020, room-rent proportionate deduction.
- Financial toxicity of cancer treatment in India, Front. Public Health, 2023 (n = 12,148).
- Catastrophic expenditure & treatment attrition, Lancet Reg. Health SE Asia, 2022;6:100058.
- IRDAI Master Circular on Health Insurance Business, 2024.

**SAFE BY CONSTRUCTION** every field is user-confirmed · outputs are labelled estimates, never guarantees · no medical advice, money only · no identifiable patient data